Sanaul Haque

Austin_Jones_Proposal

Assignment 1066

Document Details

Submission ID
trn:oid:::3117:634931439

Submission Date
Oct 6, 2026, 6:44 PM GMT+6

Download Date
Oct 6, 2026, 6:52 PM GMT+6

File Name
Austin_Jones_Proposal.pdf

File Size
182.6 KB

7 Pages

2,205 Words

12,406 Characters

20% Overall Similarity

The combined total of all matches, including overlapping sources, for each database.

Match Groups

- **14** Not Cited or Quoted 14%
  Matches with neither in-text citation nor quotation marks
- **2** Missing Quotations 1%
  Matches that are still very similar to source material
- **5** Missing Citation 6%
  Matches that have quotation marks, but no in-text citation
- **0** Cited and Quoted 0%
  Matches with in-text citation present, but no quotation marks

Top Sources

17% Internet sources
16% Publications
18% Submitted works (Student Papers)

Match Groups

- **14** Not Cited or Quoted 14%
  Matches with neither in-text citation nor quotation marks
- **2** Missing Quotations 1%
  Matches that are still very similar to source material
- **5** Missing Citation 6%
  Matches that have quotation marks, but no in-text citation
- **0** Cited and Quoted 0%
  Matches with in-text citation present, but no quotation marks

Top Sources

17% Internet sources
16% Publications
18% Submitted works (Student Papers)

Top Sources

The sources with the highest number of matches within the submission. Overlapping sources will not be displayed.

1	Publication	Chenghui Wu, Ya Sun. "Discursive Construction of Message Credibility for Chines... 2%
2	Student papers	University of Melbourne on 2026-06-01 2%
3	Student papers	University of Northumbria at Newcastle on 2026-08-24 2%
4	Internet	rifl.unical.it 2%
5	Internet	cahaya-ic.com 2%
6	Student papers	Syracuse University on 2022-05-02 1%
7	Publication	Xiaolin Shen, Tao Shen, Diwei Zheng, Ying Li. "The Influence of Felt Trust on Nurs... 1%
8	Internet	ijere.iaescore.com 1%
9	Internet	helvia.uco.es 1%
10	Internet	www.mdpi.com 1%

11 Publication

Tsuyoshi Okuhara, Hiroko Okada, Rie Yokota, Takahiro Kiuchi. "Effectiveness and ... <1%

12 Publication

Siqi Wang. "Not just in, but how: Leading physicians' engagement strategies and ... <1%

13 Internet

www.koreascience.or.kr <1%

14 Student papers

Dublin City University on 2025-08-01 <1%

15 Student papers

Liberty University on 2026-08-22 <1%

16 Publication

Blackstock, Oni J., Chinazo O. Cunningham, Lorlette J. Haughton, Ruby Y. Garner, ... <1%

17 Publication

Mimi Ngo, Emilija Jensen, Maurice Meade. "Exploring experiences of orthognathi... <1%

18 Student papers

University of Bolton on 2026-04-27 <1%

19 Student papers

University of Derby on 2022-03-18 <1%

20 Publication

Pei Duan, Zhao Liu, Song Chen, Wei Liang, Ping Hou, Ling Qian, Wei Wang, Yongbi... <1%

IARCO RESEARCH PROPOSAL

Full Legal Name: Austin Jones

Institution: The Oakridge School

Category: Senior

Class/Grade/Year: Grade 12

Country: United States

Major (if senior): Health

Submission Date: October 3, 2026

Registered Email Address: austinjjones027@gmail.com

Research Topic: Trust in Medical Pathways: Comparing Health Information Trust Among Middle School, High School, and University Students

Research Problem

The rapid expansion of social media has created a largely unregulated channel through which adolescents increasingly access health information, raising concerns about how it shapes their trust in professional healthcare. It remains unclear how perception of online health sources varies across middle school, high school, and university students.

This study will address this by examining these three groups. Middle schoolers are still developing the skills used to judge whether a source is reliable, and many have not begun managing their own health. High schoolers use social media more and have more independence, but still have limited experience with the healthcare system. University students are generally more educated, more used to handling their own medical care, and often better at evaluating health information. Because these groups differ so much in age, education, and social media use, it is not safe to assume they trust online and professional sources in the same way. It is also unclear whether trust in online sources and trust in doctors compete or simply coexist, and whether that relationship stays the same or shifts from one stage to the next.

A common assumption is that middle schoolers are unexposed to social media. However, a majority of United States children own a smartphone by age 11 [1]. For this reason, the study does not assume how much any group is exposed to social media or how skilled its members are at reading online health content. Instead, it measures both directly so that it can separate their effects from age and education. The goal is to map how trust in online sources and in doctors forms and changes across middle school, high school, and university, and to show where efforts to reduce health misinformation are most likely to help.

Literature Review

Trust in medical contexts is a multidimensional construct built on competence and honesty, and it has been linked to long-term engagement with healthcare [2], [3]. In digital environments, trust functions differently. It operates as a cognitive shortcut driven by factors such as popularity, aesthetics, and emotional connection rather than clinical credibility [4], [5].

Individuals who feel dismissed or disrespected by providers show diminished trust in professional care and are more likely to seek health information elsewhere [6]. Adolescence compounds this dynamic. It is a formative period during which independent health behaviors begin to develop, and social media plays a central role in shaping identity and information consumption [7], [8]. Together, these factors create a fragmented trust landscape in which accessibility and perceived authenticity frequently compete with medical accuracy, further complicated by the volume of misinformation circulating in digital spaces [7], [9].

Adolescents are especially susceptible to this shift because social media encourages emotional rather than analytical processing. Limited access to professional care heightens the effect. Many adolescents avoid medical visits because of cost, discomfort, or perceived judgment, which makes free and easy-to-understand online content a natural alternative [6], [9]. Perceived discrimination or feeling misunderstood by a provider can further diminish trust and push adolescents toward other sources [3]. The result is that adolescents often turn to online communities for autonomy and encouragement, yet that same independence places them in an

unregulated space where qualified experts are difficult to distinguish from convincing but untrained creators [9], [10].

Digital and eHealth literacy also vary with age and sociodemographic factors, and higher literacy supports more critical evaluation of online content [11], [12]. A majority of United States children own a smartphone by age 11, and 69% own one by age 12, so exposure to online content begins well before high school [1]. Self-reported confidence in judging sources does not always match measured ability, and that gap between perceived and actual skill may itself shape trust [4], [11]. The eHealth Literacy Scale (eHEALS) is a validated measure of perceived skill [12], so measuring the gap also requires a performance-based task. Research also separates stated attitudes from real choices, since intentions and actual decisions can differ [13]. Most existing work, however, studies a single age group. This study addresses that gap by comparing three developmental and educational stages using one common survey.

Research Question

This study investigates how trust in social media health information and professional healthcare varies across educational stages, and how the two forms of trust relate within each group. Specifically:

1. How do trust in social media health information and trust in professional healthcare differ among middle school, high school, and university students?
2. Do trust in social media and trust in doctors operate as independent constructs or as competing ones, and does that relationship strengthen or weaken with age and education?
3. How does self-reported confidence in identifying credible online health content relate to social media trust across the three groups, and does measured skill on a credibility-rating task moderate this relationship?
4. In vignette-based scenarios, does the tendency to defer to professional medical advice change across developmental and educational stages?
5. How do first-choice information source and doctor-avoidance behaviors vary across groups, and how do they relate to trust in each pathway?

Hypothesis

Social media trust will be lowest in middle schoolers, rise in high schoolers as exposure increases, and decline in university students as eHealth literacy and critical analysis skills grow. Within each group, trust in social media and trust in doctors are expected to operate as largely independent constructs.

Methodology

This study uses a cross-sectional, comparative mixed-methods survey design. It measures trust in each pathway alongside social media exposure and digital health literacy, so the three groups can be compared on the same terms.

Participants

Three groups will be recruited: middle school students (grades 6–8, roughly ages 11–14), high school students (grades 9–12, roughly ages 14–18), and university undergraduates (roughly ages 18–22). The study will recruit about 200 participants per group, for a total of about 600.

Recruitment will use stratified sampling across several schools and institutions, including public and private settings. This design improves generalizability and doesn't limit the study to a specific institution or economic region.

Survey Makeup

The survey has six sections: (Su.1) demographics and pre-screening, (Su.2) trust in social media health information, (Su.3) trust in professional healthcare, (Su.4) vignette-based decision scenarios, (Su.5) information navigation behaviors, and (Su.6) open-ended questions. Likert responses are converted to a linear 0.00–1.00 trust score toward either social media or doctors. This gives a single comparable scale across all three groups. Three further measures are included: a social media exposure and usage index; the validated eHealth Literacy Scale (eHEALS), which captures self-rated skill [12]; and a short credibility-rating task in which participants judge a set of accurate and misleading health posts, which captures actual skill. Together, they separate perceived confidence, measured skill, and exposure. Wording will be checked for reading level in the middle school group.

Data Analysis

Trust scores will be compared across the three groups using analysis of variance (ANOVA), which tests whether the average trust scores differ from one group to the next. Because age and education overlap, a regression will also examine group, social media exposure, and eHealth literacy together, so the effect of each one can be seen more clearly. The open-ended answers (Su.6) will be grouped by theme to find the common reasons behind students' source preferences.

Ethical Considerations and Approvals

This study spans several institution types, and they do not all require the same approvals. Private schools do not always require formal Institutional Review Board (IRB) review; administrative review can be sufficient. Public schools and universities are different. University research must be approved by each institution's IRB before recruitment begins. Public school districts often require their own research review or district-level approval. Minors in the middle and high school groups will need parental or guardian consent plus participant assent. University participants are adults and will give their own informed consent.

The survey collects no names, contact details, medical records, or other identifiers, and all responses stay anonymous and voluntary. Student data handling will follow applicable education-privacy rules, such as FERPA in United States schools. The eHealth Literacy Scale is a published scale, and it will be used with proper attribution and any permission required.

Expected Outcome and Significance

This study should produce the first map of how trust in medical pathways develops from middle school through university. By separating age, education, exposure, and literacy, it can show whether the relationship between social media trust and trust in doctors is consistent across life stages. Results are expected to show which developmental stage benefits most from social media literacy. If younger students trust online health information before they can critically analyze it, then teaching digital literacy early may matter more than limiting their social media use. Either way, the findings would give schools, doctors, and public health workers a clearer idea of when and how to step in against health misinformation.

Potential Limitations

This study has a few limitations. Age and education tend to go together, so it is hard to tell whether differences come from getting older or from receiving more education. University students are also adults who chose to go to college, which may make them different from the younger groups in ways the study cannot fully account for. The survey also looks at different students at one point in time, not the same students as they grow up, so it shows differences between groups rather than change within a person. All of the answers are self-reported, so students may not always share their real habits or trust. Finally, collecting data from many schools could introduce differences in access to doctors and technology that are hard to control for.

Conclusion

This proposal examines health-information trust across three developmental and educational stages: middle school, high school, and university. By measuring exposure and eHealth literacy alongside stated trust and vignette behavior, it aims to show how trust in doctors and in social media forms and diverges as young people grow up. A clearer developmental picture would help educators, clinicians, and public health workers place interventions where they can do the most against health misinformation.

Bibliography

[1] V. Rideout and M. B. Robb, "The Common Sense census: Media use by tweens and teens, 2019," Common Sense Media, San Francisco, CA, USA, 2019. [Online]. Available: https://www.commonsensemedia.org/about-us/news/press-releases/the-common-sense-census-media-use-by-tweens-and-teens-new-research. Accessed: Oct. 2026.

[2] R. C. Mayer, J. H. Davis, and F. D. Schoorman, "An integrative model of organizational trust," *Academy of Management Review*, vol. 20, no. 3, pp. 709–734, 1995, Q1. DOI: 10.5465/amr.1995.9508080335.

[3] J. Richmond et al., "Development and validation of the trust in my doctor, trust in doctors in general, and trust in the health care team scales," *Social Science & Medicine*, vol. 298, Art. no. 114827, 2022, Q1. DOI: 10.1016/j.socscimed.2022.114827.

[4] M. J. Metzger and A. J. Flanagin, "Credibility and trust of information in online environments: The use of cognitive heuristics," *Journal of Pragmatics*, vol. 59, pp. 210–220, 2013, Q1. DOI: 10.1016/j.pragma.2013.07.012.

[5] S. S. Sundar, "The MAIN model: A heuristic approach to understanding technology effects on credibility," in *Digital Media, Youth, and Credibility*, M. J. Metzger and A. J. Flanagin, Eds. Cambridge, MA, USA: MIT Press, 2008, pp. 73–100. DOI: 10.1162/dmal.9780262562324.073.

[6] H. K. Hardin, A. E. Bender, C. P. Hermann, and B. J. Speck, "An integrative review of adolescent trust in the healthcare provider relationship," *Journal of Advanced Nursing*, vol. 77, no. 4, pp. 1645–1655, 2021, Q1. DOI: 10.1111/jan.14674.

[7] J. L. Freeman, P. H. Y. Caldwell, and K. M. Scott, "How adolescents trust health information on social media: A systematic review," *Academic Pediatrics*, vol. 23, no. 4, pp. 703–719, 2023, Q1. DOI: 10.1016/j.acap.2022.12.011.

[8] E. Engel et al., "Social media influencers and adolescents' health: A scoping review of the research field," *Social Science & Medicine*, vol. 340, Art. no. 116387, 2024, Q1. DOI: 10.1016/j.socscimed.2023.116387.

[9] E. Denniss and R. Lindberg, "Social media and the spread of misinformation: Infectious and a threat to public health," *Health Promotion International*, vol. 40, no. 2, Art. no. daaf023, 2025, Q1. DOI: 10.1093/heapro/daaf023.

[10] E. Gabarron, S. O. Oyeyemi, and R. Wynn, "COVID-19-related misinformation on social media: A systematic review," *Bulletin of the World Health Organization*, vol. 99, no. 6, pp. 455–463A, 2021, Q1. DOI: 10.2471/BLT.20.276782.

[11] J. L. Claggett, B. Kitchens, and M. Paino, "Identifying the peripheral cues in the credibility assessment of online health information," *Information & Management*, vol. 61, no. 8, Art. no. 104037, 2024, Q1. DOI: 10.1016/j.im.2024.104037.

[12] C. D. Norman and H. A. Skinner, "eHEALS: The eHealth Literacy Scale," *Journal of Medical Internet Research*, vol. 8, no. 4, Art. no. e27, 2006, Q1. DOI: 10.2196/jmir.8.4.e27.

[13] M. Conner and P. Norman, "Understanding the intention–behavior gap: The role of intention strength," *Frontiers in Psychology*, vol. 13, Art. no. 923464, 2022, Q2. DOI: 10.3389/fpsyg.2022.923464.